جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ساميه السيد مهدي الغول
المحافظة : القاهره **الحي / المدينة :** شبرا **العنوان :** القاهره
رقم البطاقة : 27301160103207 **رقم التليفون :** 01222769898 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 16-01-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه السيد مهدي الغول
National ID: 27301160103207

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقق الشخصية



سامية
السيد مهدى الغول
١٠٩ ش احمد الصيفى
دار السلام - القاهره

٢٧٣٠١١٦٠١٠٣٢٠٧

KK8438430



٢٧٣٠١١٦٠١٠٣٢٠٧ ٢٠٢٤/٠٢
ربة منزل

انثى مسلمة متزوجة
عبدالعاطى حسن احمد زايد
١ البطاقة سارية حتى ٢٠٣١/٠٢/٢٦



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

USTAVE OUSSY

Patient Name	:الأستاذه / ساميه السيد مهدي	Request No.	: 13003441
Patient No.	: 27301160103207	Request Date	: 16/11/2025
Path No.	: SR-1654/25	Verification Date	: 01/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

(according to Nottingham grading system of breast cancer) .
IPT:
* ER: Negative (score 0/8).
* PR: Negative (score 2/8).
* HER2:Positive (score +3)
CELL BLOCK &SMEARS SHOWED REACRIVE SMEARS .
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: سامیه سید المهدی الغول	
MR. : 2476	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

A 53 yrs old female pt, diabetic & hypertensive, FH -ve. Diagnosed Rt breast IDC GII, cT4b N2 M0, ER0/8 PR2/8 Her2 3+. O/e..Rt breast UOQ mass 5x3cm with overlying skin dimpling, Rt axillary mass 3x2cm, Paget's disease. Pt was counselled on her MDT decision. For Portacath insertion, Labs & ECHO then start TCHP with regular assessment of response.

echo 11/6 60% EF today for c1 TCHP, TXT 70 mg, carboplatin AUC 5, loading dose phesgo, please add solucortef pt was preppared, add methabiogen and 1 GCSF WT HT

clinical pharmacy note : wt 86 ht 155 sa 1.92 for c1 taxotere 135 mg carbo auc 5 phesgo 1200/600

c2 taxotere carboplatin auc 5, received methabiogen AST ALT mildy elevated .. for liver support, ca 0.8 for ca supp otherwise accepted o/e fungating bleeding breast mass .. for close FU

Patient received 2 cycles of taxotere carbo PHESGO NOT YET APPROVED PROPAPLY due to old imaging new ct scan and bone scan requested

postmenop rt breast cancer ct4b n2 mo idc 2 er0/8 pr2/8 her23+ diabetic on exam rt breast uoq mass 4x4 skin dimpling nipple ulcer 3x3 rt ax ln 2x2 for c3 taxotere 135 mg carbopl auc 5 and proph neupogen ,new cts bs pending for approval of phesgo

FOR C4 TXT CARBOPLATIN, labs accepted .. for CT revison as it shows hypervascular HFL ??? previous CT free in 10/2025 pt started dx journey last year in baheya 5/2025 and then was transferred here in 10/2025, has only satarted ttt in 8/2026

ct cap 11/8/2026 compared to 27/10/2025 >> newly developed pulmonary nodules in both lungs largest 10*11mm + HFLs bone scan 31/8>> free for mdt meanwhile for c5 txt/carbo labs accepted

تحريرا في ,

التاريخ : 19-09-2026 **الوقت :**

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

Name : Samia El-Sayed Mahdi Al-Ghoul
Branch : Dokki

Date : 29-10-2025 10:34
Medical No. : 236861

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History

- A 52-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

- No ascites.

OPINION:

- **No CT evidence of hepatic or pulmonary deposits.**
- **Moderate fatty hepatomegaly.**

Dr.Sara Mahmoud, MD

Dr.Sara Ali

Dr.Sara Ali

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Samia Al Sayed Mahdy Alghool	Date : 27/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 2386/2025	Pat. ID: 132595

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported case of right breast carcinoma.

(I) Chest:

- Mosaic attenuation of both lungs denoting the presence of small airway disease.
- No parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right LOQ breast lesion ,for sonomammographic correlation.
- Bilateral elastofibroma dorsi.

(II) Abdomen and pelvis:

- The liver is moderately enlarged in size showing homogenous reduced CT density with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصري لتنمية الصحة مصر

04/08/2026

MDT FORM

Name:	Samia El-Sayed Elmahdy Al Ghoul	**MRN:**	2476
Age:	53 Y	**ID:**	27301160103207
Menopausal Sta	POST	**Phone:**	01014443097
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M:DM,HTN	**Reason Of MDT:**	NEW

Clinical Data:

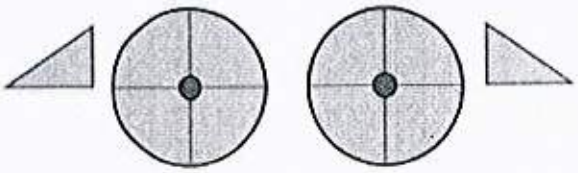
BX: RT IDC

ER: 0/8 **PR:** 2/8 **HER2:** +3 **KI67:**

Sonomammography:

RIGHT BREAST:
Right UOQ irregular spiculated dense mass measuring about 4.4 x 3.2 cm in CC view with overlying suspicious microcalcification.
• Another grouped suspicious microcalcification are seen extending anterior to the mass extending over an area measuring 4.8 cm.
Complementary U/S:
• Right 10 o'clock irregular spiculated heterogeneously hypoechoic solid mass measuring about 3.5 x 2.3 x 2.7 cm surrounded by desmoplastic reaction located 4 mm from the overlying skin.
Retracted right nipple with ulceration & skin defect.
Single indeterminate axillary (level I) lymph node measuring about 18 x 11 mm.
Single lower deep cervical prominent rounded lymph node measuring 7 x 5 mm.
CESM ACR: C. RIGHT BREAST:
(Non contrast) images shows upper outer quad mass & distortion as well as microcalcifications coinciding with site of palpable lesion
Nipple is seen ulcerated with diffuse skin thickening (6mm).
An intramammary lymph node seen in UOQ, deeply seated small this was confirmed by ultrasound as small preserved intramammary lymph node.
Recombined (Post contrast) images show :
Upper outer heterogeneously enhancing multifocal masses & non mass are seen with non grouped foci microcalcifications, the whole process is measuring 5.9 x 5.5 x 6.32 cm
Retroareolar prominent ducts with inspissated secretions extending towards the upper outer quadrant.
Complementary ultrasound confirmed the presence of multiple interconnecting masses & surrounded by similar adjacent satellite lesions ,An UOQ deeply seated IMLN
Multiple enlarged level I axillary lymph nodes with increased cortical thickness up to 6 mm? pathological .

R L



TNM

Mets Workup:

BONE SCAN:FREE
CT CAP:FREE

History:

53 YO DM, HTN IDC MULTIFOCAL WITH PAGET'S(NIPPLE ULCERATION) POOR COMPLIANCE SINCE 11-2025
CLINICALLY T4B N2 MX(CERVICAL LN PROMINENT)
DR.MAYAR FOUAD

Recommendations:

FOR NACT FOLLOWED BY RT MRM

MDT Outcome & Plan

In view of having cT4b N2 M0, Her2+ve disease:
Start NACT TCHP x6 guided by clinical response, then consider Rt MRM.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شيماء لاشين	د/ امنيه عيسوي				
8/4/2026	8/4/2026				

MDT Form
[[DOCTYPE:MDT_FORM]]

BAHEYA

Radiology Department

بهية

قسم الأشعة التشخيصية

PATIENT NAME: SAMIA ELSAYYED MAHDY ELGOEL **GENDER:** Female **DATE :** 28-May-2025 04:04 PM	**PATIENT ID:** 20225019684 **AGE:** 52 y **ORDER:** Z199120,Z199121

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast lump.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Bilateral heterogeneously dense breasts **(ACR:C)**.

RIGHT BREAST:

- Right UOQ irregular spiculated dense mass measuring about 4.4 x 3.2 cm in CC view with overlying suspicious microcalcification.
- Another grouped suspicious microcalcification are seen extending anterior to the mass extending over an area measuring 4.8 cm.

Complementary U/S:

- Right 10 o'clock irregular spiculated heterogeneously hypoechoic solid mass measuring about 3.5 x 2.3 x 2.7 cm surrounded by desmoplastic reaction located 4 mm from the overlying skin.
- Retracted right nipple with ulceration & skin defect.
- Single indeterminate axillary (level I) lymph node measuring about 18 x 11 mm.
- No U/S evidence of right infra- nor supraclavicular lymph nodes.
- Single lower deep cervical prominent rounded lymph node measuring 7 x 5 mm.

LEFT BREAST:

- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Non specific axillary lymph nodes.

OPINION:

- Right UOQ malignant mass and pathological microcalcification as above described with single indeterminate ipsilateral axillary (level I) lymph node and prominent lower deep cervical lymph node. For histopathological assessment **(BIRADS 5)**.
- Bilateral adenosis **(BIRADS 2)**.

مركز بهية الهرم
٩ شارع علوية أول شارع الهرم بجوار نادي الجزيرة

مستشفى بهية الشيخ زايد
١ الحي الأول - الشيخ زايد أمام كلية الهندسة

www.Baheya.org

16602

Mammogram

[[DOCTYPE:MAMMOGRAM]]

MRN:	20225019684
Patient Name:	SAMIA ELSAYYED MAHDY ELGOEL
D.O.B:	16/01/1973
IC No.:	27301160103207
Sex:	Female
Location:	External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

pecimen Id	992508120131	Collection Date	12/08/2025 02:40 PM
ccession No	Z-SUR25-201940	Received Date	12/08/2025 02:40 PM
ample Type	Tissue	Doctor	Abd ALLAH Mohamed Gamal

linical Information

ight breast highly suspicious lesion, BIRADS 5

ross Picture

ultiple tissue cores up to 1 cm, totally submitted

licroscopic Picture

case of
Invasive duct carcinoma , grade3
DCIS identified, intermediate nuclear grade, solid and cribriform patterns, focal necrosis
Lymphovascular invasion is identified.

rozen Diagnosis

iagnostic Summary

umour type:

Invasive duct carcinoma , grade 3

aterality

ight

pecimen type:

iopsy

ecation

istological grade

itosis score (1-3):

ubular formation score (1-3):

uclear grade (1-3):

istological grade (1-3):

iagnosis

ight breast mass, u/s guided biopsy:
Invasive duct carcinoma , grade 3

ommens

R, PR and HER2 are ordered

iopsy reporting category

5c – Invasive cancer

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name:	**SAMIA ELSAYYED MAHDY ELGOEL**	Gender:	**Female**
MRN:	**20225019584**	Ordered By:	**fatma abdelsadek ahmed abdelrahem**
DOB:	**16/01/1973** Age: **52 Y 5 M 10 D**	Order ID:	**199121**

Procedure Date: 28/05/2025 03:47:00 PM

US. Both breast

Report:

Report:

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
- Right breast lump.

TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound

REPORT:
- Bilateral heterogeneously dense breasts (ACR:C)

RIGHT BREAST:
- Right UOQ irregular spiculated dense mass measuring about 4.4 x 3.2 cm in CC view with overlying suspicious microcalcification.
- Another grouped suspicious microcalcifications are seen extending anterior to the mass extending over an area measuring 4.8 cm.

Complementary U/S:
- Right 10 o'clock irregular spiculated heterogeneously hypoechoic solid mass measuring about 3.5 x 2.3 x 3.7 cm surrounded by desmoplastic reaction located 4 mm from the overlying skin.
- Retracted right nipple with ulceration "T" skin defect.
- Single indeterminate axillary (level I) lymph node measuring about 18 x 11 mm.

No U/S evidence of right infra- nor supraclavicular lymph nodes.
Single lower deep cervical prominent rounded lymph node measuring 7 x 5 mm.

LEFT BREAST:
No spiculated mass lesions or grouped microcalcifications.
This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
Normal skin thickness and contour.
Normal nipple areola complexes.
Non specific axillary lymph nodes.

CONCLUSION:
Right UOQ malignant mass and pathological microcalcification as above described with single

Sonography
[[DOCTYPE:SONOGRAPHY]]

Patient Name:	SAMIA ELSAYYED MAHDY ELGOEL	Gender:	Female
MRN:	20225019684	Ordered By:	fatma abdelsadek ahmed abdelrahem
DOB:	16/01/1973 Age: 52 Y 5 M 10 D	Order ID:	199120

2D mammography

Procedure Date: 28/05/2025 03:47:00 PM

Report:

Report:

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
- Right breast lump.

TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound

REPORT:
- Bilateral heterogeneously dense breasts (ACR:C).

RIGHT BREAST:
- Right UOQ irregular spiculated dense mass measuring about 4.4 x 3.2 cm in CC view with overlying suspicious microcalcification.
- Another grouped suspicious microcalcification are seen extending anterior to the mass extending over an area measuring 4.8 cm.

Complementary U/S:
- Right 10 o'clock irregular spiculated heterogeneously hypoechoic solid mass measuring about 3.5 x 2.3 x 3.7 cm surrounded by desmoplastic reaction located 4 mm from the overlying skin.
- Retracted right nipple with ulceration /T skin defect.
- Single indeterminate axillary (level I) lymph node measuring about 18 x 11 mm.
- No U/S evidence of right infra- nor supraclavicular lymph nodes.
- Single lower deep cervical prominent rounded lymph node measuring 7 x 5 mm.

LEFT BREAST:
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Non specific axillary lymph nodes.

OPINION:
- Right UOQ malignant mass and pathological microcalcification as above described with single

Sonography
[[DOCTYPE:SONOGRAPHY]]

MRN:	20225019684
Patient Name:	SAMIA ELSAYYED MAHDY ELGOEL
D.O.B:	16/01/1973
IC No.:	273011601032O7
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

pecimen Id	992506150047	**Collection Date**	15/05/2025 10:47 AM
ccession No	Z-IMM25-202139	**Received Date**	15/05/2025 10:47 AM
ample Type	Paraffin Block	**Doctor**	Abd ALLAH Mohamed Gamil

mmunohistochemical Report

athology number

01940/25

linical Data

T breast cancer.

ross Picture :

eferred block submitted for ER/PR immunostaining.

ystem Used :

entana BenchMark ULTRA autostainer.

ntibody:

entana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal ntibody.

coring system :

ccording to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei osit ve for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A ample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are nmunoreactive.

lired score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, etween 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells taining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). otal score from 0 to 8.

omment:

There are much more limited data on the 2% to 1% of cancers that are low ER expressers most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from ndocrine therapy.

PR positive in 1-20% of tumor cells is considered as Low positive.

Laterality) breast Cancer:

ight

Diagnosis :

R : Negative in tumor cells (0/8).

R :Negative in tumor cells (0/8).

ignature

AHITAB IBRAHIM MOHAMED PHD

o-Signed by:

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جمهورية مصر العربية
بطاقة تحقق الشخصية



سامية
السيد مهدى الغول
١٠٩ ش احمد الصيفى
دار السلام - القاهره

٢٧٣٠١١٦٠١٠٣٢٠٧

KK8438430



٢٧٣٠١١٦٠١٠٣٢٠٧ ٢٠٢٤/٠٢
ربة منزل

انثى مسلمة متزوجة
عبدالعاطى حسن احمد زايد
١ البطاقة سارية حتى ٢٠٣١/٠٢/٢٦



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سامية
السيد مهدى الغول
١٠٩ ش احمد الصيفى
دار السلام - القاهره

٢٧٣ ٠١ ١٦ ٠١ ٣٢ ٠٧

KK8438430

National ID · Front
[[DOCTYPE:ID_FRONT]]

٢٧٣٠١١٦٠١٠٣٢٠٧ ٢٠٢٤/٠٢

ربة منزل

انثى مسلمة متزوجة

عبدالعاطى حسن احمد زايد

١ البطاقة سارية حتى ٢٠٣١/٠٢/٢٦



National ID - Back
[[DOCTYPE:ID_BACK]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

04/08/2026

MDT FORM

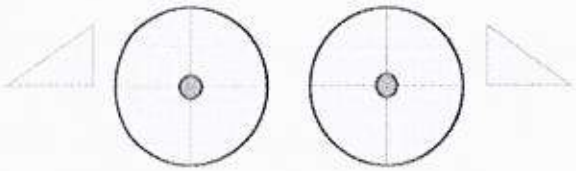
Name:	Samia El-Sayed Elmandy Al Ghoul	**MRN:**	2476
Age:	53 Y	**ID:**	27301160103207
Menopausal Sta	POST	**Phone:**	01014443097
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M.DM,HTN	**Reason Of MDT:**	NEW

Clinical Data:

BX: RT IDC

ER: 0/8 **PR:** 2/8 **HER2:** +3 **KI67:**

R L



TNM

Sonomammography:

RIGHT BREAST:
Right UOQ irregular spiculated dense mass measuring about 4.4 x 3.2 cm in CC view with overlying suspicious microcalcification.
• Another grouped suspicious microcalcification are seen extending anterior to the mass extending over an area measuring 4.8 cm.
Complementary U/S:
• Right 10 o'clock irregular spiculated heterogeneously hypoechoic solid mass measuring about 3.5 x 2.3 x 2.7 cm surrounded by desmoplastic reaction located 4 mm from the overlying skin
Retracted right nipple with ulceration & skin defect
Single indeterminate axillary (level I) lymph node measuring about 13 x 11 mm.
Single lower deep cervical prominent rounded lymph node measuring 7 x 5 mm.
CESM ACR: 5, RIGHT BREAST:
(Non contrast) images shows upper outer quad mass & distortion as well as microcalcifications coinciding with site of palpable lesion
Nipple is seen ulcerated with diffuse skin thickening (6mm).
An intramammary lymph node seen in UOQ, deeply seated small this was confirmed by ultrasound as small preserved intramammary lymph node
Recombined (Post contrast) images show :
Upper outer heterogeneously enhancing multifocal masses & non mass are seen with non grouped focal microcalcifications, the whole process is measuring 5.9 x 5.5 x 6.32 cm
Retroareolar prominent ducts with inspissated secretions extending towards the upper outer quadrant.
Complementary ultrasound confirmed the presence of multiple interconnecting masses & surrounded by similar adjacent satellite lesions .An UOQ deeply seated IMLN

Mets Workup:

BONE SCAN FREE
CT CAP:FREE

History:

53 YO DM, HTN IDC MULTIFOCAL WITH PAGET'S(NIPPLE ULCERATION) POOR COMPLIANCE SINCE 11-2025
CLINICALLY T4B N2 MX(CERVICAL LN PROMINENT)
DR.MAYAR FOUAD

Recommendations:

FOR NACT FOLLOWED BY RT MRM

MDT Outcome & Plan

In view of having cT4b N2 M0, Her2+ve disease:
Start NACT TCHP x6 guided by clinical response, then consider Rt MRM.

SIGNATURE

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY INTERNATIONAL

Patient Name	:الأستاذه / ساميه السيد مهدي	Request No.	: 13003441
Patient No.	: 27301160103207	Request Date	: 16/11/2025
Path No.	: SR-1654/25	Verification Date	: 01/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Refereing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data
Right breast mass, right axillary lymph node .

Nature of specimen
Block &slide revision .

Gross examination
Two paraffin blocks (200644-25) & (201940) and five slides were received .

Microscopic examination
* Histological type: Invasive duct carcinoma.
* Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
* DCIS: Absent
*Lobular intra epithelial neoplasia : Absent
* Angioinvasion: Absent .
* Microcalcification :Absent
*Perineural invasion: Absent
• IPT (Positive control showed appropriate reactivity) :
* ER: No nuclear reaction in tumor cells (Allred score 0/8)
* PR: Weak nuclear reaction in less than 1 % tumor cells (Allred score 2/8)
* HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
*CELL BLOCK &SMEARS :
Examination revealed lymphocytes .No malignant cells .

Diagnosis
RIGHT BREAST MASS, BLOCK AND SLIDE REVISION
-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)

Verified by :
Dr.Heba Hasan

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جمهورية مصر العربية

بطاقة تحقيق الشخصية



سامية

السيد مهدى الغول

١ ش احمد الصيفي

دار السلام - القاهره

١٩٧٣/٠٧/٢٩

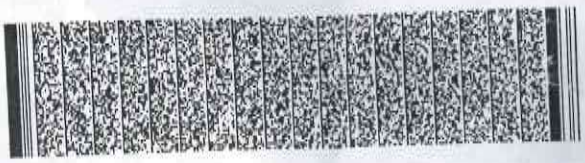
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KK8438430

National ID – Front
[[DOCTYPE:ID_FRONT]]

٢٧٣٠١١٦٠١٠٣٢٠٧ ٢٠٢٤/٠٢
ربة منزل

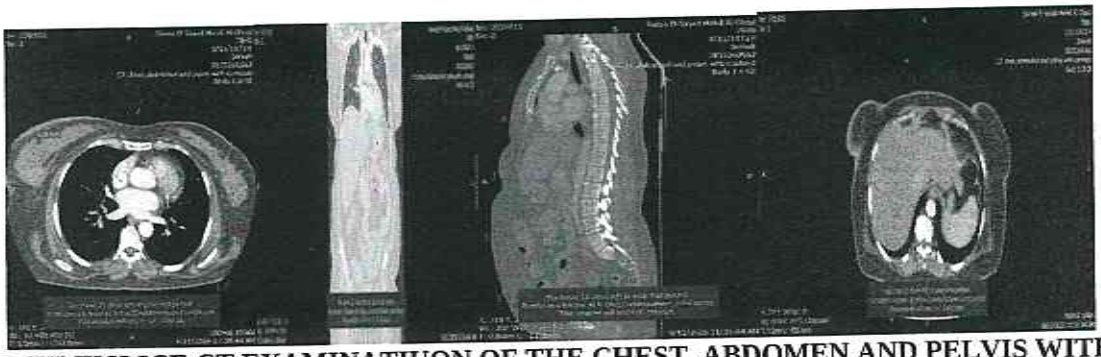
أنثى مسلمة متزوجة
عبدالعاطى حسن احمد زايد
١ البطاقة سارية حتى ٢٠٣١/٠٢/٢٦



National ID – Back
[[DOCTYPE:ID_BACK]]

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name:	**Samia El-Sayed Mahdi Al-Ghoul**
Date:	**2026-08-11**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- COMPARE TO PREVIOUS STUDY DATED 27/10/2025, THE CURRENT STUDY SHOWED

- An enhancing neoplastic mass lesion is seen in the upper outer,periareolar region of the right breast and measures about 59x43 mm in dimensions. The overlying skin is thickened, and measures about 14 mm. The nipple is retracted as well.
- Multiple variable sized pathologically-enlarged right axillary lymph nodes are seen with loss of their hilar fat contents and measures about 24mm.
- **Normal left breast.**
- **Benign-looking left axillary lymph nodes are seen with loss of their hilar fat contents and the largest one measures about 20 mm .**
- Hyperinflated both lungs.
- Newly developed enhancing variable sized tiny pulmonary nodules are seen in both lungs with the largest one seen in the anterior segment of left lower pulmonary lobe and measures about 11x10mm in dimensions.
- No patchy areas of consolidations noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- Average size of the heart with normal CT of its big vessels.
- A tiny sclerotic bony lesion is seen in DV3 and DV4.
- Average sized right lobe and enlarged left lobe of the beaver-tail liver, showing diffuse reduced parenchymal attenuation and a homogenous enhancement pattern apart from two tiny hypervascular focal lesion is seen in the anterior segment of right hepatic focal lesion(segment V111) and the largest measures about 18x16 mm in dimensions.
- No dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.4 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.

[[DOCTYPE:BONE_SCAN]]
Bone Scan

Name : Samia El-Sayed Mahdi **Date : 31-08-2026 10:07**
Branch : Dokki **Medical No. : 451046**
Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

53-year-old female with right breast carcinoma (non-operated). First bone scan. Referred for baseline skeletal staging.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Normal symmetric radiotracer distribution throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake suggestive of osteoblastic metastases.
- Renal excretion is prompt and symmetric.

Impression:

- No scintigraphic evidence of skeletal metastases.
- Follow up as clinically indicated would be recommended.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / سامیه السید مهدي	Request No.	: 13003441
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 52 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

(according to Nottingham grading system of breast cancer) ,
IPT:
* ER: Negative (score 0/8).
* PR: Negative (score 2/8).
* HER2:Positive (score +3)
CELL BLOCK &SMEARS SHOWED REACRIVE SMEARS .
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

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غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
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